

GENITAL SYMPTOMS IN A MAN

Give urgent attention to the man with genital symptoms and any of:

- Scrotal swelling/pain with any of: sudden severe pain, affected testicle higher/rotated, preceding trauma/strenuous activity: **torsion of testicle** likely
- Foreskin retracted over glans and unable to be pulled back to normal position (reduced) with swollen and very painful glans: **paraphimosis** likely
- Prolonged erection > 4 hours: **priapism** likely

Management:

- If likely **torsion of testicle** or **priapism**: refer urgently.
- If **paraphimosis** likely:
 - If glans blue/black: refer urgently.
 - If not, attempt manual reduction: wrap glans in gauze and apply increasing pressure for 10-15 minutes until foreskin can be replaced over glans. If unsuccessful, refer urgently.

Approach to the man with genital symptoms not needing urgent attention

- First assess and advise the man with genital symptoms → 49.
- Check for urethral discharge: if no visible discharge, ask patient to milk the urethra. If no urethral discharge and urinary symptoms (burning/frequency/urgency) → 59.

Urethral discharge
(with or without dysuria/burning urine)



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Treat for **male urethritis syndrome (MUS)**:

- Give single dose **ceftriaxone** 250mg IM¹ and
- Give single dose **azithromycin** 1g.
- If severe penicillin allergy², omit ceftriaxone and increase **azithromycin** to 2g.
- If partner has vaginal discharge syndrome (VDS), add single dose **metronidazole**³ 2g.
- Give partner notification slip/s with code: MUS.

Advise patient to return in 7 days if symptoms persist: ceftriaxone treatment failure likely. Refer within 7 days.

Scrotal swelling or pain



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Pain with/without swelling or discharge

Treat for **scrotal swelling (SSW)**:

- Give single dose **ceftriaxone** 250mg IM¹ and
- Give **azithromycin** 1g now and another dose of **azithromycin** 1g in 1 week.
- If severe penicillin allergy², omit ceftriaxone and increase **azithromycin** to 2g.
- Give partner notification slip/s with code: SSW.
- For pain, give **ibuprofen** 400mg 8 hourly with food for up to 5 days (avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease).
- Review after 7 days or earlier if needed: if no better, refer.

Painless lump

Exclude
Testicular cancer

Refer.

Painful, itchy or foul-smelling glans,
difficulty retracting foreskin



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If unable to retract foreskin, refer.

- If able to, retract foreskin, wash with water, dry and examine:
- If ulcer → 52.
- If glans inflamed, treat for **balanitis/ balanoposthitis (BAL)**:
 - Advise to retract and wash daily with water, avoid soap. Dry fully.
 - Give **clotrimazole cream** 12 hourly for 7 days.
 - Check urine dipstick for glucose. If glucose present, check for diabetes → 17.
 - Offer referral for medical male circumcision.
 - Advise to return if no better in 7 days:
 - If poor adherence, repeat treatment.
 - If still no better, refer.

¹For ceftriaxone 250mg IM injection: dissolve 250mg in 0.9mL **lidocaine 1%** without epinephrine (adrenaline). ²History of anaphylaxis, urticaria or angioedema. ³Advise no alcohol until 24 hours after last dose of metronidazole.